

CCP0207 – SITE OF SERVICE FOR OUTPATIENT SURGICAL PROCEDURES



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Category:	Clinical Criteria Policy
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Prior Authorization:	Prior Authorization May Apply
Plan Info:	SCAN Plan Documents
Network Coverage:	In Network; Out of Network

OVERVIEW

Outpatient surgical procedures—commonly referred to as same-day, ambulatory, or office-based surgeries—may be safely and effectively performed in a variety of clinical settings. While some facilities offer broader service capabilities, others are specifically designed to support ambulatory procedures with greater efficiency and lower cost.

Not all outpatient surgeries are appropriate for every setting. Site-of-service determinations must be based on documented clinical factors, including the nature and complexity of the procedure, the member's health status, provider privileging, and facility accessibility. Evidence-based research has demonstrated that performing outpatient procedures in lower-cost settings can result in significant savings without compromising quality of care, making this an important consideration in healthcare planning.

POLICY/POSITION STATEMENT

SCAN Health Plan recognizes that certain outpatient surgical procedures are considered safe and appropriate for performance in lower-acuity settings, including Ambulatory Surgery Centers (ASCs) and Office-Based Surgical Suites. In alignment with CMS guidance under 42 CFR §422.101(b), SCAN Health Plan may require that designated procedures be performed in these settings unless clinical documentation supports the medical necessity of a higher-acuity environment, such as a Hospital Outpatient Department (HOPD).

In accordance with [42 CFR §422.101\(b\)](#), SCAN Health Plan may apply utilization management strategies to promote care in the most clinically appropriate and cost-effective setting. SCAN encourages providers to choose the most appropriate clinical setting to ensure care that is both safe and cost-effective. We are committed to working collaboratively with providers to achieve this goal. When necessary, SCAN may redirect care to a lower-level clinical setting in coordination with providers. If a higher level of service is used repeatedly without clinical justification, SCAN may adjust reimbursement accordingly. Coverage determinations are based on the individual merits of each member's unique circumstances and are made in alignment with standards established by the Centers for Medicare & Medicaid Services (CMS) and applicable plan policies, and services must meet reasonable and necessary criteria for the diagnosis or treatment of illness or injury to be eligible for reimbursement.

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Purpose and Scope

SCAN Health Plan provides clinical guidance for redirecting select planned surgical procedures to the most appropriate site of service. This policy supports the safe, medically appropriate, and cost-effective delivery of outpatient surgical care by guiding the selection of the most suitable site of service—Office-Based Surgical Suite, Ambulatory Surgery Center (ASC) or Hospital Outpatient Department (HOPD).

Policy Application

This policy applies to all contracted and non-contracted providers submitting claims for outpatient surgical procedures for members enrolled in SCAN Medicare Advantage plans.

Review and Considerations

Elective procedures are subject to medical necessity review using procedure-specific clinical criteria regardless of the site of service. In determining the most appropriate setting for surgical care, this policy considers the individual needs of the member, the availability of services within the contracted network, the provider's credentials to perform the procedure in an ambulatory surgery center, and the geographic accessibility of such facilities in accordance with the members' plan requirements.

Applicability of ASC Site-of-Service Requirements

SCAN Health Plan applies site-of-service requirements to outpatient surgical procedures listed on the Centers for Medicare & Medicaid Services (CMS) Ambulatory Surgical Center (ASC) Covered Procedures List (CPL) for the current calendar year, as defined in 42 CFR §416.166 and published in [CMS Addendum AA](#) to the ASC Final Rule. These procedures are deemed safe and appropriate for performance in an ASC setting for Medicare beneficiaries. Reimbursement for such procedures may be contingent upon clinical review and documented medical necessity supporting the use of an alternative, higher-acuity setting.

Office Based Procedures

SCAN Health Plan supports the performance of select elective outpatient procedures in office-based surgical settings when clinically appropriate and safe. Reimbursement for procedures performed in an office setting is contingent upon documented clinical appropriateness. The following conditions may justify referral to a higher acuity setting:

- Unable to cooperate due to mental status, severe anxiety, or heightened pain sensitivity
- Prior failed attempt in the office setting due to body habitus, abnormal anatomy, or technical limitations
- Presence of a bleeding disorder that increases procedural risk
- Allergy to local anesthetic agents
- Medical conditions requiring enhanced monitoring, specialized medications, or extended recovery

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- Increased risk of complications due to severe comorbidities, such as those classified as
- ASA Physical Status III or higher.

Ambulatory Surgery Center (ASC) Based Procedures

SCAN Health Plan supports the use of Ambulatory Surgical Centers (ASCs) for elective outpatient procedures when clinically appropriate and safe, recognizing ASCs as a cost-effective alternative to hospital outpatient departments for cases not requiring advanced monitoring or complex perioperative care. However, patients presenting with severe systemic disease and functional limitation—classified as [American Society of Anesthesiologists \(ASA\) Physical Status III or higher](#)—may require a higher-acuity setting. Additionally, ASC-specific operational limitations, such as weight thresholds or exclusionary health conditions, may further restrict eligibility.

Hospital Outpatient Department Based Procedures

The outpatient hospital setting is deemed medically necessary for members presenting with any of the following indications, conditions, or comorbidities, including **but not limited to**:

- Active hepatitis or advanced liver disease (MELD Score > 8)
- Alcohol dependence or abuse
- Anticipated difficult airway
- Anticipated need for blood transfusion
- Anticipated need for overnight stay for recovery and care
- Anticipated prolonged surgical duration (> 3 hours)
- Bleeding disorders requiring replacement factors, blood products, or specialized infusions (excluding DDAVP)
- Chronic obstructive pulmonary disease (COPD)
- Coronary artery disease or peripheral vascular disease with ongoing ischemia or recent drug-eluting stent placement (within 1 year)
- End-stage renal disease with hyperkalemia or dialysis dependency
- Implanted pacemaker
- Moderate or severe reduction of ejection fraction
- Moderate to severe obstructive sleep apnea
- Morbid obesity (Body Mass Index > 40)
- Ongoing myocardial ischemia or severe valve dysfunction
- Personal or family history of anesthesia complications (e.g., malignant hyperthermia)
- Poorly controlled asthma ($FEV_1 < 80\%$ despite treatment)
- Poorly controlled diabetes
- Poorly controlled hypertension
- Recent cerebrovascular accident (CVA) or transient ischemic attack (TIA) (within 3 months)
- Recent history of substance abuse (especially cocaine)
- Recent myocardial infarction (within 3 months)
- Recent placement of bare metal stents or angioplasty (within 90 days)
- Severe valvular heart disease

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- Symptomatic cardiac arrhythmias despite medical management therapy
- Uncompensated congestive heart failure (NYHA Class III or IV)
- Uncontrolled seizure disorder

Criteria for Determining the Optimal Clinical Setting for Elective Procedures

To ensure that an elective procedure is performed in the most appropriate clinical setting, the following factors must be assessed and clearly documented in the member's medical record when applicable:

- The member's specific benefit plan
- Geographic access to in-network providers
- Availability and capability of ambulatory surgical centers (ASCs)
- Physician credentialing and privileging status
- Presence of significant comorbidities (refer to the list of qualifying conditions above)
- Classification under the American Society of Anesthesiologists (ASA) Physical Status (PS) System

If an Ambulatory Surgery Center (ASC) is determined, by the operating surgeon or the ASC itself, to lack the necessary resources to deliver safe, effective, or medically necessary surgical care in a timely manner, the procedure may be performed in an outpatient hospital setting. This determination must be clearly stated and documented in the member's medical record.

DEFINITIONS

Hospital Outpatient Departments (HOPDs)

Hospital Outpatient Departments are licensed units of an acute care hospital that provide outpatient surgical services. These settings are equipped to manage higher acuity patients and complex procedures, with access to emergency services, advanced diagnostic capabilities, and inpatient resources. HOPDs are typically reimbursed at higher rates due to their expanded infrastructure and staffing requirements.

Ambulatory Surgery Centers (ASCs)

Ambulatory Surgery Centers are freestanding facilities that operate independently of hospital systems and are licensed to perform outpatient surgical procedures. ASCs are designed to support low- to moderate-risk patients undergoing elective procedures that do not require inpatient care. These centers must meet CMS Conditions for Coverage to serve Medicare beneficiaries and are subject to state and federal regulatory oversight. ASCs offer a cost-effective alternative to hospital-based settings while maintaining high standards of safety and quality.

Office-Based Surgical Suites

Office-Based Surgical Suites are procedural areas located within a physician's office or

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clinic, where minor surgical interventions are performed under local anesthesia or minimal sedation. These settings are appropriate for low-risk procedures with limited complexity and minimal recovery needs. Regulation of office-based surgery varies by state, and these facilities may not be subject to the same licensing or accreditation standards as ASCs or HOPDs. While cost-effective and convenient, their use must be supported by clinical appropriateness and provider privileging.

American Society of Anesthesiologists Physical Status (ASA PS) Score

The American Society of Anesthesiologists (ASA) Physical Status Classification System was designed to provide clinicians with a straightforward method for categorizing a patient's physiological condition. This system aids in estimating operative risk by offering a subjective evaluation of the patient's overall health, organized into five distinct classes:

- ASA I: A normal healthy patient
- ASA II: A patient with mild systemic disease
- ASA III: A patient with severe systemic disease
- ASA IV: A patient with severe systemic disease that is a constant threat to life
- ASA V: A moribund patient who is not expected to survive without the operation
- ASA VI: A declared brain-dead patient whose organs are being removed for donor purposes

REGULATORY CITATIONS/REFERENCES

- [CMS Ambulatory Surgical Centers \(ASC\) Center](#) (Accessed 10/30/2025)
- [Medicare Claims Processing Manual. Chapter 14 - Ambulatory Surgical Center](#) (Accessed 10/30/2025)
- [Code of Federal Regulation Part 416—AMBULATORY SURGICAL SERVICES](#) (Accessed 10/30/2025)
- [CMS Ambulatory Surgical Center \(ASC\) Payment](#) (Accessed 10/30/2025)
- [Code of Federal Regulations 42 CFR §422.101\(b\)](#) (Accessed 10/30/2025)
- [Healthcare Financial Management Association: HOPDs vs. ASC: Understanding Payment Differences](#) (Accessed 10/30/2025)
- [CMS ASC Payment Rates - Addenda](#) (Accessed 10/30/2025)
- [Ambulatory Surgery Center Association \(ASCA\) Advancing Surgical Care: The ASC Cost Differential](#) (Accessed 10/30/2025)
- [The American Society of Anesthesiologists Statement on ASA Physical Status Classification System](#) (Accessed 10/30/2025)
- [Centers for Medicare & Medicaid Services \(CMS\) Place of Service Code Set](#) (Accessed 10/30/2025)
- [Section 1862\(a\)\(1\)\(A\) of the Social Security Act: Exclusions from Coverage and Medicare as Secondary Payer](#) (Accessed 10/30/2025)

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DISCLAIMERS

The information provided above is accurate as of the policy review date. However, CMS coverage policies are subject to periodic updates and revisions. As a result, this document may not reflect the most recent changes. For the latest information on applicable National Coverage Determinations (NCDs), Local Coverage Determinations (LCDs), Local Coverage Articles (LCAs), and CMS Online Manual System Transmittals, please visit the Medicare Coverage Database at: <https://www.cms.gov/medicare-coverage-database/search.aspx>.

This Policy is subject to the SCAN Medical Policy Terms of Use and Conditions, available at <https://medicalpolicy.scanhealthplan.com/terms-of-use>.

POLICY GOVERNANCE

Developed by: SCAN Medical Policy

Responsible for Implementation (Lead Department): SCAN Utilization Management (UM), Embrace, Claims, Network Management, and Medical Claims Review Departments

Maintained by: SCAN Medical Policy

Impacted Teams: UM, Embrace, Medical Claims Review, Claims, Grievance and Appeals Department (GAD), Network Management, Medical Policy, Compliance.

POLICY HISTORY

Date:	Description:	Control #:
11/20/2025	UMC Approval	1-140-11202025F
11/20/2025	Effective Date of Policy	1-140-11202025F
5/1/2026	Update template formatting, BCG number	2-207-05012026D

RELATED POLICIES

PBP0307 – Site of Service for Part B Drug Administration

RETIRED/SUPERSEDED POLICIES

BCG0140 – Site of Service for Outpatient Surgical Procedures